HEALTH-A-THON 2026 · DIABETES CARE · DOCTOR / CARE TEAM

Ask, don’t guess.

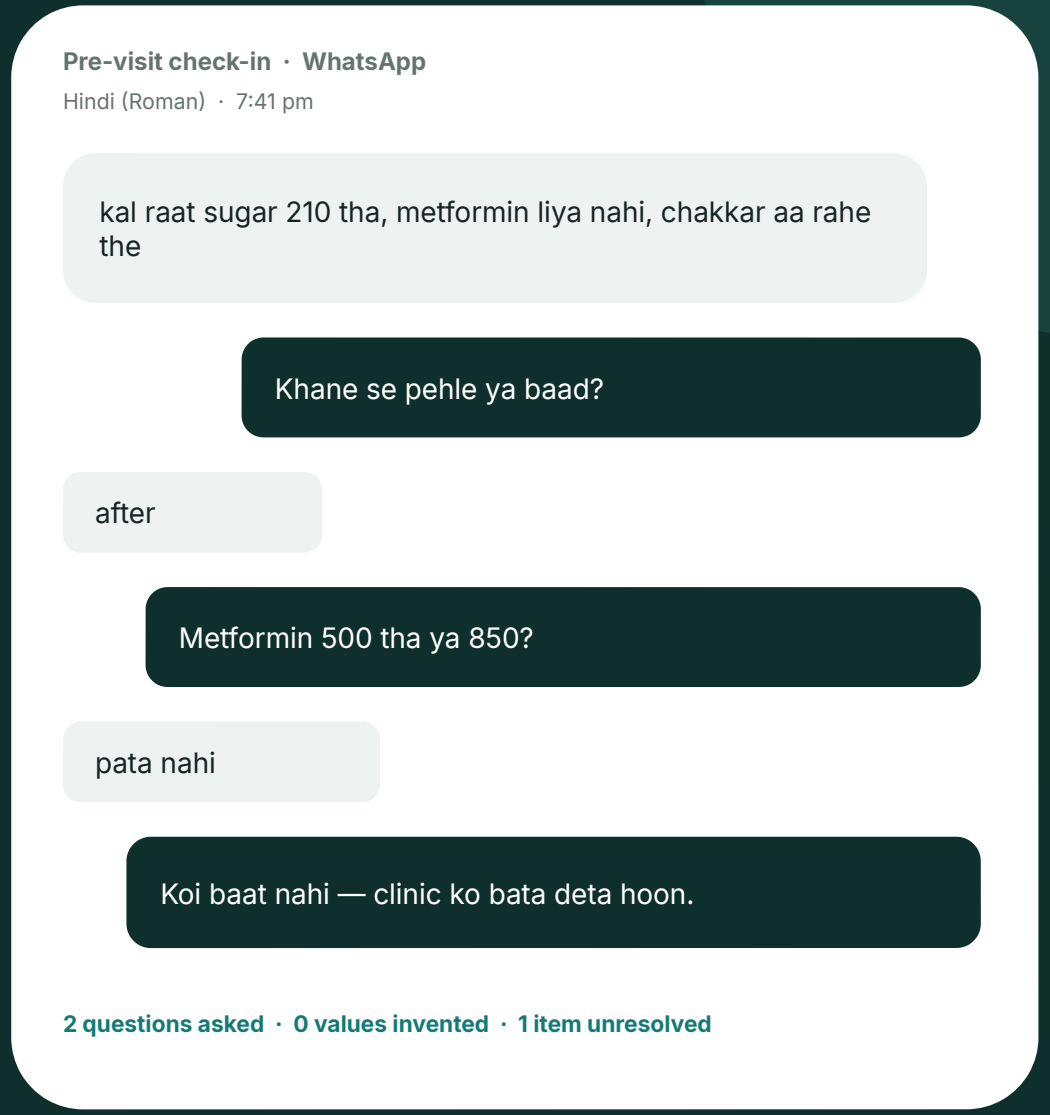
A multilingual pre-visit agent for independent diabetes clinics.

Patients and caregivers answer in their language. The agent asks when it is unsure. The clinician gets a source-linked brief — and decides.

0 guesses · 2 questions

GLUCOFLOW · PREVISIT

Synthetic data · Assistive only · Human approval required



THE PROBLEM

The 2.2-minute visit has a context problem.

The clinic sees a snapshot. The patient lives the interval.

2.2 min

average physician time per patient in nine super-specialty OPDs

IJCMH 2019

101.3M

people with diabetes in India; 136M more with prediabetes

ICMR-INDIAB 2024

1 bag

of paper reports, a 30-second story, and a patient who may not speak English

field observation



The missing context is not in the EMR. It is in the patient's language, between visits.

Source: Pandey et al., Int J Community Med Public Health 2019 (single government-hospital study, nine super-specialty clinics); ICMR-INDIAB, IJDDC 2024.

THE INSIGHT

In India, the script is a safety variable.

Same message. Different script. Different reliability.

5–12
F1 point drop on Romanized Indian-language messages

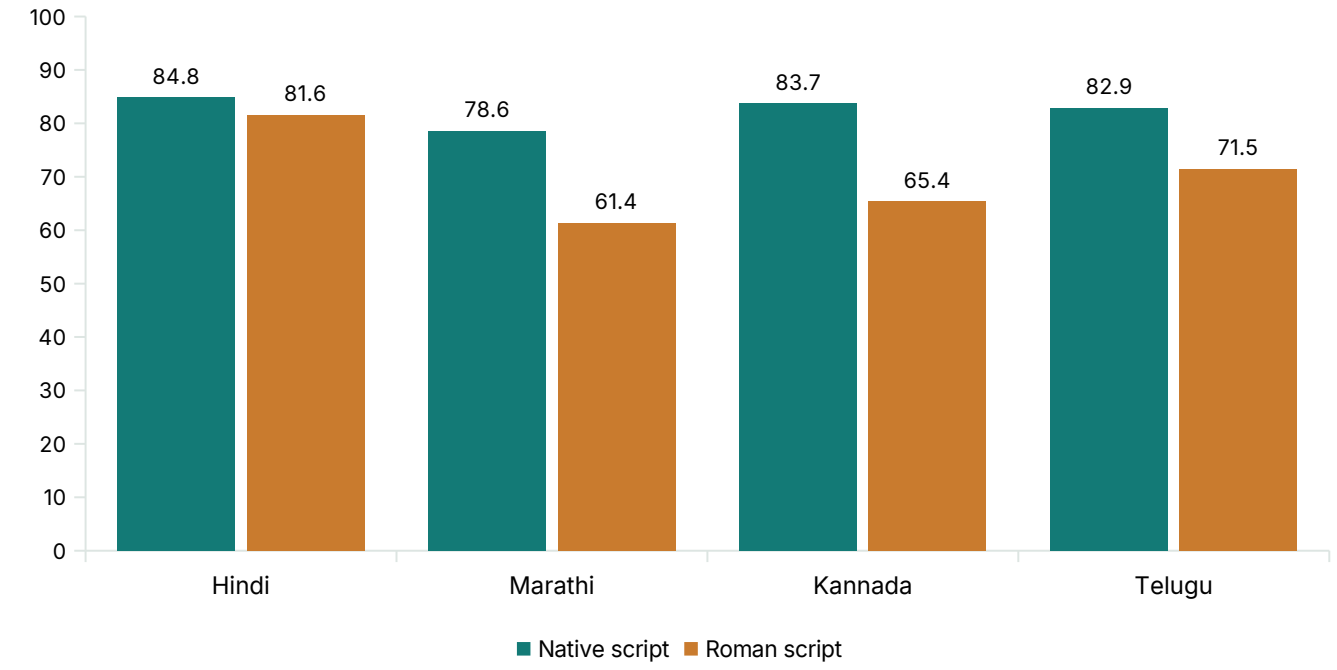
10–20
point drop for Marathi, Kannada, Telugu and Nepali

7.7%
missed emergencies on Roman script; 6.5% on native script

"Models often understand the intent, but still act on it wrongly."

F1 score by script · Claude 4.5

Higher is better. Four Indian languages, same patient messages.



Marathi: 78.6 → 61.4

Source: Script Gap, The Web Conference 2025. 133k WhatsApp conversations; 3,156 patient messages across five Indian languages and Nepali. Claude 4.5 results shown.

THE SOLUTION

Three agents. One brief. The clinician decides.

A workflow, not a chatbot. The router is deterministic code.



AUDIT & APPROVAL — Every question, answer, router decision and clinician action is logged. No record write without human approval.

The state survives every handoff: what was asked, what was said, what is uncertain, who decided.

The LLM handles language and extraction. It never decides when to skip a gate.

ASK INSTEAD OF GUESS · SHOW THE UNCERTAINTY · KEEP THE HUMAN IN CONTROL

Design vocabulary follows the ACL 2026 survey of workflow-level clinical agents: roles, handoffs, shared state, evidence-grounded tools, safety gates and audit-ready logs.

THE PRODUCT

The agent asks. The clinician decides.

A real pre-visit exchange, in the patient's language.

PATIENT · WHATSAPP

Hindi (Roman) · 7:41 pm

kal raat sugar 210 tha, metformin liya nahi, chakkar aa rahe the

Khane se pehle ya baad?

after

Metformin 500 tha ya 850?

pata nahi

Koi baat nahi — clinic ko bata deta hoon.

210 mg/dL · after dinner | Missed dose | Dizziness | Dose unknown

CLINICIAN · PRE-VISIT BRIEF

Asha R. · P0482 · 21 Oct · synthetic

CONFIRMED · 2

Glucose 210 mg/dL · after dinner
Dizziness reported

Source: 7:42 pm message · patient-reported

NEEDS CLARIFICATION · 1

Missed metformin — dose unknown
Agent asked; the patient did not know.

Routed to clinic · not guessed · not published

WHAT WE DON'T KNOW

Which metformin dose · whether dizziness continues
Whether the missed dose was morning or night

Confirm | Edit | Ask staff to follow up

Audit: 2 questions asked · 2 items confirmed · 1 item routed

0 guesses · 2 uncertainties surfaced · 1 clinician decision · audit logged

Synthetic patient and synthetic message. No diagnosis, treatment recommendation or clinical interpretation is produced.

THE OBVIOUS QUESTION

Why not Apollo 24/7?

Apollo owns the care journey. We work for the clinics it doesn’t own.

APOLLO 24/7

46M

registered users · 70+ specialties · labs · pharmacy

Clinical Intelligence Engine (Google MedLM) synthesises records for Apollo clinicians.

Patient, doctor, lab and pharmacy are all inside Apollo’s network.

Provider-owned

GLUCOFLOW PREVISIT

1 link

one brief · any clinic · no EMR replacement

Collects the missing context in the patient’s language — native or Roman script.

Asks when uncertain; audits the refusal; the clinician decides.

Clinic-agnostic

Apollo synthesises the record it already owns. We collect the context no one has — and show what we don’t know.

Apollo 24/7: 46M registered users, 70+ specialties, health records, labs and pharmacy (Apollo Pharmacy, Jul 2026). Clinical Intelligence Engine built with Google MedLM (Google Cloud, Jan 2024). Public evidence describes it as Apollo's platform; no clinic-agnostic offering found.

SAFETY & SCOPE

Assistive by design. Auditable by default.

The scope is narrow on purpose.

WE NEVER

- ✕ Diagnose or interpret symptoms
- ✕ Recommend or change treatment
- ✕ Provide clinical decision support
- ✕ Score clinical risk
- ✕ Interpret values or reference ranges
- ✕ Act autonomously

WE ALWAYS

- ✓ Label every item patient-reported
- ✓ Require clinician approval before the record
- ✓ Route fixed red-flag phrases to staff, not AI triage
- ✓ Log actor, time, reason and source
- ✓ Fail closed: ask, route, or fall back to the form
- ✓ Keep the original words next to every value

“A person can always override automation, and important actions leave an audit trail.” — Health-a-thon scope

IRL 3–4: monitored pilot, not autonomy. The red-flag path is a fixed administrative escalation to a human, not a clinical triage decision.

THE EVIDENCE

We don’t claim accuracy. We measure wrong guesses.

A small test now, a 60–90 day pilot against a simple form.

NOW

10-scenario test

4 native script · 4 Roman script · 2 code-mixed
3 ambiguous medicines · 2 missing timings
2 conflicting readings · 1 red-flag · 2 caregiver

MEASURE

Extraction accuracy · clarification rate · resolution rate
wrong-guess rate · human-review rate · time

Target: 0 wrong guesses

A 10-scenario test is not clinical validation. It is enough to show the router behaves as designed.

PILOT

60–90 days · one clinic

30–50 patients · 2–3 clinicians
Arm A: simple structured form. Arm B: Glucoflow PreVisit.

PRIMARY METRICS

Completion · time-to-brief · item completeness
clinician edits · caregiver participation · comprehension

PROPOSED SUCCESS CRITERIA

≥60% completion · ≤3 min brief · ≤15% edits
0 wrong guesses · 0 safety events

Caution from the evidence: mDiabetes (1,048 adults, rural Karnataka) found AI-personalised WhatsApp messages did not beat static messages. Our baseline is a simple form, not “no tool”.

Sources: Script Gap (Web Conference 2025); mDiabetes, JMIR 2025; AI-hospitals survey, ACL 2026; agentic AI review, IEEE Access 2026.

THE ASK

Everyone is building AI that answers.
We built the agent that knows when to ask.

One diabetes clinic. 30–50 patients. 60–90 days. A doctor partner for the question set.

WHY NOW

Agents, multilingual LLMs and WhatsApp are finally good enough. But AI alone isn't enough — the workflow and the human gate are the product.

WHY US

Clinic-agnostic by design: a deterministic uncertainty router, a source-linked brief and an audit trail — grounded in the Script Gap research, not a model demo.

WHAT WE NEED

One clinic and a doctor partner. 30–50 patients due for follow-up. 60–90 days to measure against a simple structured form.

Pilot: one clinic · 30–50 patients · 60–90 days

Synthetic data · Assistive only · No diagnosis or treatment advice · Human approval required